Common Sense Initiative

Mike DeWine, *Governor*
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Business Impact Analysis

Agency, Board, or Commission Name: DBH

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Regulation/Package Title (a general description of the rules' substantive content):

Service Rules

Rule Number(s): 5122-29-01, 03, 06, 07, 08, 09.1, 10, 12, 13, 15, 16, 17, 18, 19, 22, 27, 28, 29

Date of Submission for CSI Review: October 7, 2026

Public Comment Period End Date: October 14, 2026

Rule Type/Number of Rules:

New/___ rules
No Change/____ rules (FYR? ___)
Amended/__18__ rules (FYR? ___)
Rescinded/____ rules (FYR? ___)

The Common Sense Initiative is established in R.C. 107.61 to eliminate excessive and duplicative rules and regulations that stand in the way of job creation. Under the Common Sense Initiative, agencies must balance the critical objectives of regulations that have an adverse impact on business with the costs of compliance by the regulated parties. Agencies should promote transparency, responsiveness, predictability, and flexibility while developing regulations that are fair and easy to follow. Agencies should prioritize compliance over punishment, and to that end, should utilize plain language in the development of regulations.

Reason for Submission

1. **R.C. 106.03 and 106.031 require agencies, when reviewing a rule, to determine whether the rule has an adverse impact on businesses as defined by R.C. 107.52. If the agency determines that it does, it must complete a business impact analysis and submit the rule for CSI review.**

   Which adverse impact(s) to businesses has the agency determined the rule(s) create?

   The rule(s):

   a. ☒ **Requires a license, permit, or any other prior authorization to engage in or operate a line of business.**

   b. ☒ **Imposes a criminal penalty, a civil penalty, or another sanction, or creates a cause of action for failure to comply with its terms.**

   c. ☐ **Requires specific expenditures or the report of information as a condition of compliance.**

   d. ☐ **Is likely to directly reduce the revenue or increase the expenses of the lines of business to which it will apply or applies.**

Regulatory Intent

2. **Please briefly describe the draft regulation in plain language.**
   Please include the key provisions of the regulation as well as any proposed amendments.

   These rules regulate community behavioral health services regulated by the Department of Behavioral Health.

3. **Please list the Ohio statute(s) that authorize the agency, board or commission to adopt the rule(s) and the statute(s) that amplify that authority.**

   R.C. 5119.36 and R.C. 5119.34

4. **Does the regulation implement a federal requirement? Is the proposed regulation being adopted or amended to enable the state to obtain or maintain approval to administer and enforce a federal law or to participate in a federal program?**
   If yes, please briefly explain the source and substance of the federal requirement.

   No

5. **If the regulation implements a federal requirement, but includes provisions not specifically required by the federal government, please explain the rationale for**

exceeding the federal requirement.

N/A

6. **What is the public purpose for this regulation (i.e., why does the Agency feel that there needs to be any regulation in this area at all)?**

ORC 5119.35 and 5119.36 required the department to license community mental health and addiction providers for the purpose of protecting the health and safety of the patients and the public.

7. **How will the Agency measure the success of this regulation in terms of outputs and/or outcomes?**

The Department will be monitoring mental health and addiction providers for issues necessitating changes in license term and working to minimize those events.

8. **Are any of the proposed rules contained in this rule package being submitted pursuant to R.C. 101.352, 101.353, 106.032, 121.93, or 121.931?**
If yes, please specify the rule number(s), the specific R.C. section requiring this submission, and a detailed explanation.

No

Development of the Regulation

9. **Please list the stakeholders included by the Agency in the development or initial review of the draft regulation.**
If applicable, please include the date and medium by which the stakeholders were initially contacted.

DBH had a comment period from September 11-18, 2026, where stakeholders could submit written comments. A stakeholder meeting with the Coalition for Healthy Communities was held and DBH participated in an ODM meeting for stakeholder associations to review rules on October 6, 2026.

We reviewed all comments and made changes to the rules where we determined were important prior to their submission to CSI.

10. What input was provided by the stakeholders, and how did that input affect the draft regulation being proposed by the Agency?

Themes of stakeholder comments:

1. Clarification and alignment of definitions: adding or aligning the definition of substance use disorder across rules, clarifying terms such as "co-occurring conditions," "collateral contact," and "self-management skills." Changes were made to ensure consistency and reduce ambiguity across behavioral health rules.
2. Restoration of existing or prior language for accuracy: restoring references to resilience, recovery, and wellness in peer support rules, re-including assessment, referral, monitoring and follow-up in SUD case management. Maintain continuity with existing regulatory frameworks and avoid unintended narrowing of services.
3. Removal of ambiguous, duplicative, or confusing language: removing references to MRSS/adult crisis services where necessary, eliminating terms like "skilled treatment," which are not standard. Streamline rules to enhance clarity and operation usability.
4. Updating service requirements to match practice and policy needs: conditioning prescriber availability on whether a program offers medication management, allowing SUD case management to occur in group settings, updating expectations for peer-run organizations, including hours of operation and funding clarity. Modernize service standards and reduce unnecessary regulatory burden.
5. Enhancing cross-rule and cross-agency consistency: adding references to additional background checks guided by DCY rules, clarifying citation issues and cross-rule references. Promote uniformity across interconnected behavioral health systems.
6. Broadening access and eligibility where appropriate: clarifying that individuals with co-occurring or secondary conditions remain eligible for ACT. Reduces unnecessary eligibility limitations and ensure inclusive service access.

11. What scientific data was used to develop the rule or the measurable outcomes of the rule? How does this data support the regulation being proposed?

Data collected from licensed providers showing changes in service outcomes can help to inform the department in rule development.

We also engaged with Medicaid to help inform the department regarding the rules by understanding the data Medicaid has collected.

12. What alternative regulations (or specific provisions within the regulation) did the Agency consider, and why did it determine that these alternatives were not appropriate? If none, why didn't the Agency consider regulatory alternatives? ***Alternative regulations may include performance-based regulations, which define the required outcome, but do not dictate the process the regulated stakeholders must use to***

comply.

DBH did not consider alternative regulations. R.C. 5119.25 authorizes the DBH Director to adopt rules specifying the mental health services and alcohol and drug addiction services that are required to be certified under R.C. 5119.36.

13. What measures did the Agency take to ensure that this regulation does not duplicate an existing Ohio regulation?

The Department worked with the Ohio Department of Medicaid to ensure that these rules were consistent with Medicaid program integrity rules.

14. Please describe the Agency's plan for implementation of the regulation, including any measures to ensure that the regulation is applied consistently and predictably for the regulated community.

Once the rules are final filed, DBH will engage in stakeholder education regarding the rules. DBH's Office of Licensure and Certification will provide technical assistance to providers as the changes are implemented. Rules are applied consistently during this Office's surveys conducted prior to initial certification, renewal of certification, and at other times DBH is authorized to conduct surveys.

Adverse Impact to Business

15. Provide a summary of the estimated cost of compliance with the rule(s). Specifically, please do the following:

a. Identify the scope of the impacted business community, and

The impacted business community is any community behavioral health service provider that desires to provide the new service.

b. Quantify and identify the nature of all adverse impact (e.g., fees, fines, employer time for compliance, etc.).

The adverse impact can be quantified in terms of dollars, hours to comply, or other factors; and may be estimated for the entire regulated population or for a representative business. Please include the source for your information/estimated impact.

The primary impact is through the fee for certification. However, providers' employees will also spend time ensuring compliance with the rules changes.

16. Are there any proposed changes to the rules that will <u>reduce</u> a regulatory burden imposed on the business community? Please identify. ***(Reductions in regulatory burden may include streamlining reporting processes, simplifying rules to improve readability,***

eliminating requirements, reducing compliance time or fees, or other related factors).

No

17. Why did the Agency determine that the regulatory intent justifies the adverse impact to the regulated business community?

R.C. 5119.25 authorizes the DBH Director to adopt rules specifying the mental health services and alcohol and drug addiction services that are required to be certified under R.C. 5119.36.

Requiring providers of services to be certified and are providing actual scientific, sound services protects patients and the public. In addition, many of the changes are being done at the request of the Ohio Department of Medicaid to meet new program integrity requirements.

Regulatory Flexibility

18. Does the regulation provide any exemptions or alternative means of compliance for small businesses? Please explain.

No

19. How will the agency apply Ohio Revised Code section 119.14 (waiver of fines and penalties for paperwork violations and first-time offenders) into implementation of the regulation?

DBH works with each provider to resolve issues that do not impact health and safety, as well as first-time offenses where wrongful intent is not present. Regarding the latter, DBH treats these instances as an opportunity to educate providers about best practices.

20. What resources are available to assist small businesses with compliance of the regulation?

DBH's Office of Licensure and Certification provides technical assistance regarding compliance with all rules in O.A.C. Chapter 5122-29.